TRIFINITY EMS | CLINICAL MANUAL

PURPOSE OF THIS MANUAL

This manual exists to help EMS personnel create high-quality, believable, and engaging scene work scenes in and out of the operating room. Medical scene work is used as a tool for scene work, not as a barrier. No prior medical knowledge is required to use this guide.

In Trifinity EMS, you are not just responding to injuries — you are creating the medical narrative. You are given an injury type (GSW, stab, burn, blunt force trauma) and from that, you build the scene: assessment, escalation, complications, treatment, surgery, and recovery.

This guide teaches you how to think, act, and improvise like EMS, while keeping scenes fair, immersive, and fun for everyone involved.

CORE EMS SCENE WORK PRINCIPLES

1. Injuries are prompts, not scripts
   - A single injury can have many outcomes
   - Severity is determined through scene work, not assumptions
2. Complications create scene work
   - Complications add tension and depth
   - They should never feel like punishment
3. Pacing matters
   - Minor injuries = short scenes
   - Major trauma = long, multi-stage scenes
4. Consent and communication matter
   - EMS should coordinate scene work when scenes become long or intense
   - scene work should never trap or frustrate players

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SECTION 1: FIRST CONTACT & SCENE ASSESSMENT

Initial Arrival Checklist

When arriving on scene, EMS should always scene work the following:

- Secure the scene (ensure no ongoing danger)
- Introduce yourself and your role
- Establish patient consciousness
- Ask for the patient's name
- Ask where it hurts
- Check pulse and breathing
- Observe visible injuries

scene work Prompt Examples

Use questions to extract scene work from the patient:

- "Are you feeling dizzy or lightheaded?"
- "Can you take a deep breath for me?"
- "Do you feel numbness anywhere?"
- "Does the pain feel sharp, dull, or burning?"

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SECTION 2: PAIN, SHOCK, AND SEDATION

Recognizing Shock

Shock is a progressive condition, not instant death.

Early Shock

- Pale skin
- Anxiety or agitation
- Increased heart rate

Compensated Shock

- Rapid breathing
- Weak pulse
- Confusion

Decompensated Shock

- Loss of consciousness
- Extremely low blood pressure
- Organ failure risk

Shock may worsen during transport or surgery.

Pain Management Levels

- Mild Pain Relief: For minor injuries, patient remains fully conscious
- Moderate Sedation: For painful procedures, patient semi-aware
- Heavy Sedation / Anesthesia: For surgery or unsafe patients

Heavy sedation carries risks:

- Airway compromise
- Blood pressure drops
- Need for assisted breathing

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SECTION 3: INJURY SEVERITY SCALING

Every injury should be placed into a severity tier:

Minor

- Stable vitals
- No organ involvement
- Minimal bleeding
- Short treatment and discharge

Moderate

- Ongoing bleeding
- Possible fractures or tissue damage
- Requires monitoring or surgery

Critical

- Organ damage
- Severe bleeding
- Loss of consciousness
- High risk of complications

Severity may change during scene work.

SECTION 4: INJURY-SPECIFIC SCENE WORK GUIDES

Gunshot Wounds (GSW)

Possible Presentations

- Grazing wound
- Through-and-through
- Lodged bullet
- Bone shattering impact

Possible Complications

- Internal bleeding
- Infection
- Nerve damage
- Organ perforation
- Collapsed lung

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Surgical scene work Flow (GSW)

1. Control bleeding
2. Explore wound
3. Remove bullet/fragments
4. Repair tissue or organs
5. Drain placement
6. Closure and monitoring

Stab Wounds

Possible Presentations

- Superficial puncture
- Deep penetration
- Retained object

Possible Complications

- Hidden arterial bleeding
- Organ puncture
- Infection

Surgical scene work Flow (Stab)

1. Assess depth and trajectory
2. Control bleeding
3. Remove object (if safe)
4. Repair damage
5. Close wound

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Burn Injuries

Burn Severity

- First-degree: Pain and redness
- Second-degree: Blisters and swelling
- Third-degree: Skin loss and numbness

Possible Complications

- Infection
- Fluid loss
- Shock

Surgical scene work Flow (Burn)

1. Debridement
2. Infection control
3. Skin grafting (if required)
4. Long-term wound care

Blunt Force Trauma

Common Causes

- Vehicle accidents
- Falls
- Assaults

Possible Complications

- Internal bleeding
- Fractures
- Brain injury

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Surgical scene work Flow (Blunt Force)

1. Imaging and assessment
2. Stabilization
3. Surgical repair if needed
4. Monitoring for delayed symptoms

SECTION 5: SURGERY SCENE WORK STRUCTURE

Surgery should be scene worked in clear stages:

1. Pre-op briefing and consent
2. Sedation and preparation
3. Incision and exploration
4. Discovery of complications
5. Intervention and repair
6. Closure
7. Transfer to recovery

SECTION 6: POST-OPERATIVE & LONG-TERM SCENE W

Post-op scene work keeps EMS relevant:

- ICU monitoring
- Follow-up appointments
- Infection checks
- Reduced mobility scene work
- Rehabilitation
- Scar and trauma scene work

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SECTION 7: EMS CREATIVITY & FAIR PLAY RULES

EMS May:

- Add reasonable complications
- Adjust severity for scene work pacing
- Coordinate scene work for long scenes

EMS May Not:

- cause immediate fatal outcomes
- Add unrealistic complications
- Use medicine as punishment
- God-mod outcomes

SECTION 8: RANDOM COMPLICATION TABLES

Gunshot Wound Complications

1. Internal bleeding worsens
2. Bullet fragmented on impact
3. Bone splintering discovered
4. Nerve damage causing loss of movement
5. Infection risk due to contamination
6. Collapsed lung (torso wounds)
7. Organ perforation
8. Blood pressure instability
9. Delayed bleeding resumes
10. No complication discovered

Stab Wound Complications

1. Hidden arterial bleed
2. Organ puncture
3. Retained foreign material
4. Infection signs appear
5. Wound track deeper than expected
6. Internal bleeding
7. Loss of sensation near wound
8. No complication discovered

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Burn Injury Complications

1. Severe fluid loss
2. Infection develops
3. Skin necrosis
4. Pain escalation
5. Shock progression
6. Failed graft attempt
7. Reduced mobility
8. No complication discovered

Blunt Force Trauma Complications

1. Internal bleeding
2. Undetected fracture
3. Brain injury symptoms
4. Organ bruising
5. Delayed collapse
6. Swelling increases
7. Nerve compression
8. Blood pressure drop
9. Respiratory distress
10. No complication discovered

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SECTION 9: QUICK SCENE BUILDER CHEAT SHEET

Step 1: Injury Type

- GSW / Stab / Burn / Blunt Trauma

Step 2: Initial Severity

- Minor / Moderate / Critical

Step 3: Assessment

- Consciousness / Pain level / Breathing / Bleeding

Step 4: Escalation

- Choose or roll complication / Adjust severity if needed

Step 5: Treatment Path

- Stabilization / Surgery or monitoring / Recovery plan

Step 6: Outcome

- Full recovery / Limited mobility / Follow-up care / Long-term effects